

PAINFIX PHYSIOTHERAPY · INDIVIDUALIZED REHABILITATION

From assessment to individualized rehabilitation

Move from a broad pain label to a shared, testable rehabilitation plan: screen, understand, select, dose, review.

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01 · ADJUNCT, NOT THE PLAN

Manual therapy can open a window for movement

Use it as an adjunct for short-term symptom modulation or movement facilitation when examination suggests it may help the person engage with active rehabilitation. Explain options, limits and alternatives.



Choose deliberately

Select the technique for a clear clinical reason, not as a standalone cure.



Keep consent active

Check response, preferences and whether the technique is earning its place.



Return to activity

Use the window to practise movement and build confidence.

> “Hath se treatment madad kar sakta hai, lekin recovery ka bunyadi hissa active plan hota hai.”

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02 · START WHERE SUCCESS IS POSSIBLE

Exercise works best when it is individualized

Offer a progressive programme matched to goals, capability, preferences and access. No single exercise is best for everyone.



Goal-led

Choose exercise that supports the person’s goals and fits their capability, preferences and access.



Repeatable

Start at a level the person can repeat and build from.



Progressive

Temporary symptom fluctuation does not automatically mean harm; use response over time to guide dosage.

> “Jo exercise aap ke liye mumkin aur pasand ho, usi se shuru karke dheere dheere barhate hain.”

Roman Urdu explanation for patient conversations.

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03 · DOSAGE IS A CLINICAL DECISION

Strengthen for the person’s life, not a perfect score

Set load, repetitions, sets, frequency and recovery at a manageable starting dose. Progress one variable at a time when tolerance is stable.

Repeatable

A dose the person can complete and recover from.

Challenging

A manageable increase when tolerance is stable.

Functional

Strength that transfers to lifting, stairs, work, caregiving or sport.

Adapt for flare-ups, fatigue, comorbidity and confidence.

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Match mobility and motor control to the person

Use mobility work, motor-control practice and graded exposure when they address the person’s impairments, goals or feared activities.



Explore

Find a supported movement the person can tolerate.



Vary

Build options rather than enforcing one perfect posture.



Transfer

Progress toward the real task and keep decisions shared.

> “Har jism ka movement pattern mukhtalif ho sakta hai; maqsad behtar function hai, perfect posture nahi.”

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05 · MAKE PROGRESS VISIBLE

Walking and aerobic activity make progress visible

Choose a repeatable baseline, such as time, distance or daily opportunities. Increase gradually using tolerance and recovery, not a rigid calendar.

01

Baseline

Choose a repeatable time, distance or daily opportunity.

02

Repeat

Build consistency; use tolerance and recovery to guide activity.

03

Small progression

Increase gradually, one manageable step at a time.

04

Review

Track longer walks, easier errands and more participation.

Guidance-informed pathway · NICE NG59.

Attach activity to routines and track wins that matter.

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Education turns advice into self-management

Shared decision-making keeps the plan realistic and adaptable. Agree on a small number of actions the person believes they can do.



Prepare

Discuss pacing, sleep, activity, stress, flare-up planning and when to seek review.



Plan for flare-ups

Agree how rest, movement and review will work when symptoms change.



Revisit barriers

Adjust without blame, using what the person learns about their response.

> “Flare-up ka matlab hamesha nuksan nahi; plan mein aaram, movement aur review ka tareeqa pehle se rakhein.”

Roman Urdu explanation for patient conversations.

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07 · PARTICIPATION IS THE TARGET

Return to work and daily function by graded exposure

Break the task into tolerable steps and progress toward the work or life demand that matters.

Make confidence a clinical outcome, not an afterthought.

Scale the task

Break the task into tolerable steps: duration, load, pace, complexity or environment.

Maintain participation

Modify tasks temporarily where useful, while preserving participation and confidence.

Progress to the demand

Review what the person can do now, then progress toward the work or life demand that matters.

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08 · REASSESS, DO NOT REPEAT AUTOMATICALLY

Measure what matters, then reassess the plan

Review meaningful change, adherence, barriers and adverse responses to choose the next step. Track pain alongside function, personal goals and confidence.



Symptom

Pain intensity and adverse responses.



Function

Disability measures such as ODI or RMDQ.



Goal

Walking, sitting, lifting, sleep, work or participation.



Confidence

What feels possible now.



Next step

Adjust the plan using adherence and barriers; do not repeat automatically.

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Clinical pathway and key takeaways

1 Screen and assess

2 Agree goals

3 Select active rehabilitation

4 Use indicated adjuncts

5 Progress exposure and participation

6 Measure and review

Keep the principles visible

1. Care is individualized, active and collaborative; no single technique is a universal cure.
2. Use guidance to inform decisions, then assess the person in front of you.
3. Educational overview only: not original clinical research and not a substitute for individual assessment.

Selected guidance

WHO. Guideline for non-surgical management of chronic primary low back pain in adults in primary and community care settings. 2023.

NICE. Low back pain and sciatica in over 16s: assessment and management, NG59.

JOSPT/APTA. Clinical practice guideline for low back pain; consult the current edition for detailed recommendations.

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